In both cases, the venous valves can be patent. This is also one of the important reasons to combine the classical physical examination with technical investigations, especially duplex ultrasound techniques. The duplex examination is therefore best performed by the physician together with a technician.

Symptomatology

The venous leg ulcer arises either 'spontaneously' or often after a minor trauma. The patient's complaints due to the ulcer may vary from mild to severe. Although many textbooks state otherwise, venous leg ulcers can indeed be painful. Pain is particularly prominent during the ulcerative phase of white atrophy or when additional factors such as infection are present.

Clinically, venous leg ulcer is part of chronic venous insufficiency (CVI). Patients with CVI develop various skin changes over time, including pigmentation, corona phlebectatica, white atrophy, and dermato- and liposclerosis. The exact percentage of patients who progress to symptomatic disease remains unknown, as this has never been adequately studied.

The venous ulcer is typically located on the medial side of the ankle, less commonly on the lateral side. A specific variant is ulceration in acroangiodermatitis of the forefoot, seen in patients with foot pump incompetence. If an ulcer is located elsewhere on the lower leg, non-venous causes should be strongly suspected.

The clinical features of CVI are well recognized. For completeness, they are listed here: varicosities, edema, corona phlebectatica, hyperpigmentation, dermato- and liposclerosis, white atrophy, secondary lymphedema, stasis dermatitis (more accurately termed eczema cruris in the context of CVI), and ulceration. Skin changes in venous insufficiency result from alterations in both macro- and microcirculation. It remains unclear why extensive dermato- and liposclerosis develop in some patients while others predominantly exhibit white atrophy. Local factors may contribute and require further investigation.

Quality of Life

Venous ulcers significantly impact patients' lives and affect multiple domains of health-related quality of life (HRQOL), including bodily pain, health perception, mental health, social functioning, and vitality. Treatment and, in particular, healing of venous ulcers lead to marked improvements in these areas.

Although several disease-specific HRQOL instruments have been developed, studies have shown them to be suboptimal. Generic instruments such as the SF-36, SF-12, and EuroQoL-5D are currently recommended for assessing the impact on patients' lives. Today, healthcare systems increasingly require treatment evaluation that includes patient-reported outcomes.

Chapter 2 – Diagnostics

Introduction

The differential diagnosis of venous leg ulcer is broad. However, only a few conditions are commonly encountered. Establishing an accurate diagnosis promptly is essential, as different underlying causes require entirely different management strategies. Misdiagnosis can have serious consequences for the patient.

Anamnesis

Scientific basis

A thorough anamnesis is indispensable. Many patients with leg ulcers have complex medical histories and comorbidities. However, high-quality studies evaluating the diagnostic value of specific anamnestic items are lacking.

Conclusion

| Level 4 | Good studies on the value of specific items for anamnesis are not available. |